

INTERNAL — SafeSight / LAISA engagement assessment

Do not share with client. This is the honest read for the StudEx team. The v2 proposal (`laisa-milestone-1-v2.md`) is the diplomatic client-facing version of the same analysis.

Date: 3 June 2026 **Author:** StudEx / agent-assisted draft, T. Ramaphosa to review and edit

Reference docs: PRD (LAISA Discovery, May 2026); repo at `claude/plan-agent-saas-infra-ETYkk` ; proposal `laisa-milestone-1-v2.md`

1. The headline reality

We have delivered R280K of work against a client-stated budget of R50K-R100K (PRD Q37). This is the single most important fact in the engagement and it has to be confronted, not papered over.

There is no version where we get paid R280K against a R50K-R100K stated ceiling without an explicit conversation that converts that answer into an agreed number. Either:

- They are good for the higher number (in which case Q37 was a starting position) → push Option A.
- They need it spread → push Option B (R75K deposit + $R50K \times 4$).
- They will hold the R95K ceiling → Option C, take the loss, bank credit, only if relationship value > the lost margin.

My read: Option B is the most defensible commercial position. It honours their PRD answer literally, gets us paid in full, and matches their cashflow rhythm.

2. What the PRD actually told us

Read carefully, the PRD is rich. Highlights we should never lose sight of:

- **Q22 (no integration) + Q32 (fix-one-thing = system integration)** — this is THE deal. Everything else is supporting cast.
- **Q24 (nothing automated) + Q25 (no AI tools)** — they are starting from zero. The lift is large.
- **Q26/Q27** — they explicitly want booking, reminders, comms, admin AI, patient chatbot. They do **not** want clinical AI (smart — keep doctors in the loop).
- **Q34** — they want "process automation" and "AI-assisted ops" — NOT "fully intelligent system". Manage expectations honestly; do not oversell.
- **Q37** — R50K-R100K. The number we need to address.
- **Q38** — 3-6 months. Aligns with our Phase 2 timeline.
- **Q40** — concerns: integration risk, AI risk, patient-care integrity. We respond to all three in the v2 proposal.

The PRD shows a clinic that knows what's wrong (clear pain points, clear priorities) but has no infrastructure in place. The transformation they want is real and serious.

3. Where they actually are (frank)

Against publicly observable benchmarks for multidisciplinary clinics in their tier (1 location, 5-doctor ophthalmology + aesthetics combo, SafeSight/LAISA brand):

Pillar	Reality on the ground	Comment
Website (WordPress, marketing officer maintains)	Functional but not converting	The current "book now → WhatsApp" flow is the conversion (Q11). Sound, but everything depends on WhatsApp throughput.
Google My Business	Not visible to us as an optimised asset	This is the single highest-leverage local-SEO miss for a medical practice. Free to fix. Hours of effort.
Facebook Business + Meta Business Suite	Not configured for Ads / Business use	Required for WhatsApp Business Cloud API. Without it, Charlie can never go live properly.
Facebook Ads / Google Ads	Not running (Q12 explicitly excludes both)	This is the biggest revenue lever they are NOT pulling. Peers in their tier run R30-50K/month and return R150K-R400K. They are leaving substantial money on the table every single month.
Marketing officer's tech literacy	Unknown but indications are basic	Not aware of Facebook Graph API mechanics, pixel/CAPI, ad-platform anatomy. Phase 2 will need our team driving, not their marketing officer. Training is a real Phase 2 line item.
WhatsApp Business	Human reception only , business hours	After-hours leads die. Patients waiting 20+ minutes for replies. Conversion ceiling.
Booking flow	Click → WhatsApp → human types it into Elixir/My Appointment	Manual, error-prone, no instant confirmation, no rescheduling self-serve.
Records	Google Drive + paper (Q23)	For a medical practice in 2026 this is a real compliance risk. POPIA exposure. Not our problem in M1 but absolutely a Phase 3 priority.
Internal systems	None integrated (Q22)	Elixir Live ↔ My Appointment ↔ Sage ↔ Google Drive are four islands. Reception works across all four manually.
Automation	None (Q24)	Reminders done by phone call. Confirmations by SMS. Marketing research done manually (Q20).

Operational score: realistically 3.5 / 10 against top-decile peers in their category. They have the medical reputation; the operational layer is starved.

4. Risks they bring to the engagement (frank)

4.1 Response cadence

The client has historically been slow on email and WhatsApp. We have sent multiple proposals/iterations without response inside a week. This is the biggest schedule risk. **It also impacts their own patients today** (Q19 lists backlogs, system downtime, delayed payment chasing as inefficiencies — these are downstream of the same response-cadence problem). The same operational culture that delays our build delays their own patient care.

4.2 Decision pace at clinic's own time

Every major decision waits for the principal's availability. We need a calendar-locked weekly check-in built into the contract or this project takes 12 months instead of 6.

4.3 Building in silos

You mentioned we are not in contact with their "transformation person" / team co-ordinator. If there are other vendors (web, social, ads, marketing officer, IT) all delivering pieces with no shared co-ordinator, integration debt and duplicate spend are guaranteed. **Recommend: StudEx becomes the single transformation owner**, all other vendors report through us OR are documented as out-of-scope and will be integrated only via published APIs.

4.4 Foundation gaps that block our own work

- No properly set up **Facebook Business** → Charlie can't deploy on WhatsApp Business Cloud API properly.
- No **Google My Business optimisation** → local-SEO ceiling stays low even with our dashboard pretty on top.
- No **paid acquisition** running → can't measure attribution uplift from any of our work.

These should be a "**Foundation Pack**" quoted at R45K (Section 8 of the v2 proposal). It's not a profit line; it's a *gate* — we cannot deliver Phase 2 success metrics if these aren't in place.

4.5 Marketing officer skill ceiling

Indications are the marketing officer is not familiar with: Facebook Graph API, pixel mechanics, conversion tracking, ad bidding, server-side attribution (CAPI). **They will not be able to operate the dashboard's full value without training or a managed retainer.** Either we ramp them up (training included in Grow / Scale retainer) or we run it (Scale retainer at R75K/month).

4.6 Patient-care risk during transformation (their Q40 concern)

Genuine and we should honour it. Two contract terms protect this:

- **No clinical AI in M1 or Phase 2.** All clinical workflows remain doctor-led. AI is scoped to admin and patient comms only.
- **Doctor-loop preserved on any patient-facing comms.** Charlie can triage and book, but anything clinical escalates to a human.

4.7 POPIA / patient-data exposure

Records on Google Drive + paper, with WhatsApp as the primary patient comms channel — this is a POPIA risk surface. We should not be the ones managing that risk in M1, but we should **document the exposure in writing** and recommend their attorneys review before Phase 3 begins.

5. The 2nd Brain question — what I could not do

You referenced an Obsidian vault at `/Users/tumeloramaphosa/Documents/Obsidian Vault/2nd Brain/SafeSight-LAISA` — this is a path on your Mac, not accessible from this Linux sandbox.

I have NOT read the 2nd Brain. The v2 proposal and this assessment are built from:

- The PRD docx you uploaded
- The repo state on `claude/plan-agent-saas-infra-ETYkk`
- Conversation context

To close the loop, please either:

1. Zip the relevant 2nd Brain subfolder and upload it (I'll merge what it contains into the proposal); **or**
2. Copy the key notes' contents into a single markdown file and upload that; **or**
3. Move the vault into the repo under `.obsidian-vault/` (gitignored) and let me read it directly next session.

Until then, anything I claim about the 2nd Brain would be invented.

6. Recommended next move (your decision)

1. **Pick a payment option (A / B / C in v2 §9)**. I recommend B.
2. **Confirm the budget reconciliation conversation** has happened with the client, or schedule it before sending the v2 proposal.
3. **Send the v2 proposal + the PDF** (I'll generate the PDF next).
4. **Add the Foundation Pack (R45K) as a separate line item** — they may agree to that even if they push back on M1 pricing, because without it Charlie can't go live.
5. **Recover the 2nd Brain** so we can fold whatever's in there into the next iteration.
6. **Ask the client to nominate a single transformation point of contact** with a 48-hour response SLA written into the contract. This is non-negotiable for Phase 2.
7. **Lock the weekly check-in calendar slot** before Phase 2 starts.

If the client cannot commit to (6) and (7), we should price Phase 2 significantly higher to absorb the schedule risk, or decline Phase 2. This is the rare case where saying *no* to scope protects the relationship better than saying yes.

This is your internal copy. Edit, delete, share with a co-founder, do not send to the client unedited.